

Body dysmorphic disorder

Body dysmorphic disorder (**BDD**), occasionally still called **dysmorphophobia**, is a mental disorder characterized by the obsessive idea that some aspect of one's own body part or appearance is severely flawed and therefore warrants exceptional measures to hide or fix it.^[1] In BDD's delusional variant, the flaw is imagined.^[2] If the flaw is actual, its importance is severely exaggerated.^[2] Either way, thoughts about it are pervasive and intrusive, and may occupy several hours a day, causing severe distress and impairing one's otherwise normal activities. BDD is classified as a somatoform disorder, and the *DSM-5* categorizes BDD in the obsessive–compulsive spectrum, and distinguishes it from anorexia nervosa.

BDD is estimated to affect from 0.7% to 2.4% of the population.^[2] It usually starts during adolescence and affects both men and women.^{[2][3]} The BDD subtype muscle dysmorphia, perceiving the body as too small, affects mostly males.^[4] Besides thinking about it, one repetitively checks and compares the perceived flaw, and can adopt unusual routines to avoid social contact that exposes it.^[2] Fearing the stigma of vanity, one usually hides the preoccupation.^[2] Commonly unsuspected even by psychiatrists, BDD has been underdiagnosed.^[2] Severely impairing quality of life via educational and occupational dysfunction and social isolation, BDD has high rates of suicidal thoughts and attempts at suicide.^[2]

Body dysmorphic disorder	
Other names	Body dysmorphia, dysmorphic syndrome, dysmorphophobia
	
A cartoon of a patient with body dysmorphia looking in a mirror, seeing a distorted image of himself	
Specialty	<u>Psychiatry</u> , <u>clinical psychology</u>

Contents
<u>History</u>
<u>Signs and symptoms</u>
<u>Causal factors</u>
<u>Social media</u>
<u>Diagnosis</u>
<u>Treatment</u>
<u>Medication and psychotherapy</u>
<u>Self-improvement</u>
<u>References</u>
<u>External links</u>

History

In 1886, Enrico Morselli reported a disorder that he termed *dysmorphophobia*, which described the disorder as a feeling of being ugly even though there does not appear to be anything wrong with the person's appearance.^{[5][6]} In 1980, the American Psychiatric Association recognized the disorder, while categorizing it as an atypical somatoform disorder, in the third edition of its *Diagnostic and Statistical Manual of Mental Disorders (DSM)*.^[3] Classifying it as a distinct somatoform disorder, the DSM-III's 1987 revision switched the term to *body dysmorphic disorder*.^[3]

Published in 1994, DSM-IV defines BDD as a preoccupation with an imagined or trivial defect in appearance, a preoccupation causing social or occupational dysfunction, and not better explained as another disorder, such as anorexia nervosa.^{[3][7]} Published in 2013, the DSM-5 shifts BDD to a new category (*obsessive-compulsive spectrum*), adds operational criteria (such as repetitive behaviors or intrusive thoughts), and notes the subtype muscle dysmorphia (preoccupation that one's body is too small or insufficiently muscular or lean).^[8]

The term "dysmorphic" is derived from the Greek word, 'dismorfia' - meaning abnormal or apart, and 'morpho' meaning shape. Morselli described people who felt a subjective feeling of ugliness as people who were tormented by a physical deficit. Sigmund Freud (1856-1939), the Austrian founder of psychoanalysis, once described his patients as a "Wolf Man," as he was experiencing classical symptoms of BDD. The person he was referring to was a wealthy Russian aristocrat named Sergei Pankejeff (1886-1979).^[9]

Signs and symptoms

Dislike of one's appearance is common, but individuals with BDD have extreme misperceptions about their physical appearance.^[10] Whereas vanity involves a quest to aggrandize the appearance, BDD is experienced as a quest to normalize the appearance merely.^[2] Although delusional in about one of three cases, the appearance concern is usually non-delusional, an overvalued idea.^[3]

The bodily area of focus can be nearly any and is commonly face, hair, and skin.^[11] In addition, multiple areas can be focused on simultaneously.^[2] A subtype of body dysmorphic disorder is bigorexia (anorexia reverse or muscle dysphoria). In muscular dysphoria, patients perceive their body as excessively thin despite being muscular and trained.^[6] Many seek dermatological treatment or cosmetic surgery, which typically do not resolve the distress.^[2] On the other hand, attempts at self-treatment, as by skin picking, can create lesions where none previously existed.^[2]

BDD is an obsessive-compulsive disorder^[12] but involves more depression and social avoidance despite DOC.^[1] BDD often associates with social anxiety disorder.^[13] Some experience delusions that others are covertly pointing out their flaws.^[2] Cognitive testing and neuroimaging suggest both a bias toward detailed visual analysis and a tendency toward emotional hyper-arousal.^[14]

Most generally, one experiencing BDD ruminates over the perceived bodily defect several hours daily or longer, uses either social avoidance or camouflaging with cosmetics or apparel, repetitively checks the appearance, compares it to that of other people, and might often seek verbal reassurances.^{[1][2]} One might sometimes avoid mirrors, repetitively change outfits, groom excessively, or restrict eating.^[11]

BDD's severity can wax and wane, and flareups tend to yield absences from school, work, or socializing, sometimes leading to protracted social isolation, with some becoming housebound for extended periods.^[2] Social impairment is usually greatest, sometimes approaching avoidance of all social activities.^[11] Poor

concentration and motivation impair academic and occupational performance.^[11] The distress of BDD tends to exceed that of either major depressive disorder or diabetes, and rates of suicidal ideation and attempts are especially high.^[2]

Causal factors

As with most mental disorders, BDD's cause is likely intricate, altogether biopsychosocial, through an interaction of multiple factors, including genetic, developmental, psychological, social, and cultural.^{[15][16]} BDD usually develops during early adolescence,^[11] although many patients note earlier trauma, abuse, neglect, teasing, or bullying.^[17] In many cases, social anxiety earlier in life precedes BDD. Though twin studies on BDD are few, one estimated its heritability at 43%.^[18] Yet other factors may be introversion,^[19] negative body image, perfectionism,^{[15][20]} heightened aesthetic sensitivity,^[16] and childhood abuse and neglect.^{[16][21]}

Social media

Constant use of social media and “selfie taking” may translate into low self-esteem and body dysmorphic tendencies. The sociocultural theory of self-esteem states that the messages given by media and peers about the importance of appearance are internalized by individuals who adopt others’ standards of beauty as their own.^[22] Due to excessive social media use and selfie taking, individuals may become preoccupied about presenting an ideal photograph for the public.^[23] Specifically, females’ mental health has been the most affected by persistent exposure to social media. Girls with BDD present symptoms of low self-esteem and negative self-evaluation. Researchers in Istanbul Bilgi University and Bogazici University in Turkey found that individuals who have low self-esteem participate more often in trends of taking selfies along with using social media to mediate their interpersonal interaction in order to fulfill their self-esteem needs.^[24] The self-verification theory, explains how individuals use selfies to gain verification from others through likes and comments. Social media may therefore trigger one's misconception about their physical look. Similar to those with body dysmorphic tendencies, such behavior may lead to constant seeking of approval, self-evaluation and even depression.^[25]

In 2019 systematic review using Web of Science, PsycINFO, and PubMed databases was used to identify social networking site patterns. In particular appearance focused social media use was found to be significantly associated with greater body image dissatisfaction. It is highlighted that comparisons appear between body image dissatisfaction and BDD symptomatology. They concluded that heavy social media use may mediate the onset of sub-threshold BDD.^[26]

Individuals with BDD tend to engage in heavy plastic surgery use. Mayank Vats from Rashid Hospital in the UAE, indicated that selfies may be the reason why young people seek plastic surgery with a 10% increase in nose jobs, a 7% increase in hair transplants and a 6% increase in eyelid surgery in 2013.^[27] In 2018, the term “Snapchat Dysmorphia” was brought to life after several plastic surgeons reported that some of their patients were seeking plastic surgeries to mimic “filtered” pictures.^[28] Filtered photos, such as those on Instagram and Snapchat, often present unrealistic and unattainable looks that may be a causal factor in triggering BDD.^[29]

Diagnosis

Estimates of prevalence and gender distribution have varied widely via discrepancies in diagnosis and reporting.^[1] In American psychiatry, BDD gained diagnostic criteria in the DSM-IV, having been historically unrecognized, only making its first appearance in the DSM in 1987, but clinicians' knowledge

of it, especially among general practitioners, is constricted.^[30] Meanwhile, shame about having the bodily concern, and fear of the stigma of vanity, makes many hide even having the concern.^{[2][31]}

Via shared symptoms, BDD is commonly misdiagnosed as social anxiety disorder, obsessive-compulsive disorder, major depressive disorder, or social phobia.^{[32][33]} Social anxiety disorder and BDD are highly comorbid (within those with BDD, 12–68.8% also have SAD; within those with SAD, 4.8-12% also have BDD), developing similarly in patients -BDD is even classified as a subset of SAD by some researchers.^[34] Correct diagnosis can depend on specialized questioning and correlation with emotional distress or social dysfunction.^[35] Estimates place the Body Dysmorphic Disorder Questionnaire's sensitivity at 100% (0% false negatives) and specificity at 92.5% (7.5% false positives).^[36] BDD is also comorbid with eating disorders, up to 12% comorbidity in one study. Both eating and body dysmorphic disorders are concerned with physical appearance, but eating disorders tend to focus more on weight rather than one's general appearance.^[37]

BDD is classified as an obsessive-compulsive disorder in DSM-5. It is important to treat people with BDD as soon as possible because the person may have already been suffering for an extended period of time and as BDD has a high suicide rate, at 2-12 times higher than the national average.^{[10][37]}

Treatment

Medication and psychotherapy

Anti-depressant medication, such as selective serotonin reuptake inhibitors (SSRIs), and cognitive-behavioral therapy (CBT) are considered effective.^{[11][38][39]} SSRIs can help relieve obsessive-compulsive and delusional traits, while cognitive-behavioral therapy can help patients recognize faulty thought patterns.^[11] Before treatment, it can help to provide psychoeducation, as with self-help books and support websites.^[11]

Self-improvement

For many people with BDD cosmetic surgery does not work to alleviate the symptoms of BDD as their opinion of their appearance is not grounded in reality. It is recommended that cosmetic surgeons and psychiatrists work together in order to screen surgery patients to see if they have BDD, as the results of the surgery could be harmful for them.^[40]

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